

PSYCHOSOCIAL RISK ASSESSMENT — INTEGRATED PACKAGE — SWITCHBOARD SERVICES, LOGAN HOSPITAL

Risk report · hazard matrix · proposed controls · assessment methodology · scope · conduct of the assessment · recommended assessor

Cory Lea Shepherd, AO3 Switchboard Services · August 2026 · Prepared under the *Work Health and Safety Act 2011* (Qld), the *WHS Regulation 2011* (Qld), the *Managing the risk of psychosocial hazards at work* Code of Practice 2022, and consistent with ISO 45003. This document consolidates, as one integrated package, the worker's contribution offered under ss 47–49 for the clause 7.2 assessment: the risk report and hazard matrix, the proposed controls, the proposed methodology, the matters proposed for scope, the proposed conduct of the assessment, and providers available to perform it. Every hazard is described as a feature of the work and its systems; the 'existing controls' column records the position as known to me as an operator of the room; nothing in this document is a demand or an allegation against any person.

PART 1 — FRAMEWORK

Section 19 of the Act requires the health of workers — expressly including psychological health — to be ensured so far as reasonably practicable. The Regulation requires psychosocial hazards to be identified, the risks assessed, controls implemented in the order of the hierarchy, and controls reviewed, in consultation with affected workers (ss 47–49). The Code supplies the recognised hazard categories, the assessment factors (duration, frequency and severity of exposure), and the data sources. Risk ratings below apply the Code's factors; where a category of harm has already materialised in a diagnosed psychological injury within the room, the consequence limb is treated as established rather than speculative.

PART 2 — THE HAZARD MATRIX (framework applied)

#	HAZARD (Code)	HOW IT PRESENTS	EXISTING CONTROLS IDENTIFIED	REQUIRED BY	RISK (duration × frequency × severity)	PROPOSED CONTROL
1	High job demands	200–300 calls/shift, many distressed or urgent; multiple concurrent systems; emergency codes throughout; absorbed functions when support absent	None identified: no demand-based staffing standard, no surge protocol, no occupancy bound	s 19; Reg (identify/assess/control); Code — job demands	HIGH — sustained (years) × daily × injury of this kind has occurred	Evidence-based rostering (P7); escalation/backup pathways (P1, P4); occupancy standard (scope (e))
2	Low job control	Operational changes effective without operator consultation; no operator input to roster design	None identified: no consultation step precedes operational change	ss 47–49 (consultation is mandatory, not optional); Code	HIGH — the Code's high-risk demands/control combination, sustained	Consultation before change (P2); real-time team consultation in supervisor decisions (P6); roster input (P7)
3	Poor support	Supervision and decision-support not reliably available in real time; after-hours and weekend work continues unchanged	None identified after hours: no on-shift supervision structure; business-hours management only	s 19; Code — support	HIGH — daily after-hours exposure, years	Integrated manager in hours (P3); 24/7 rotating on-shift supervisor (P6); structured handover (P8)
4	Low role clarity	Functions accrete without documented boundaries (complaints, departmental support, roster administration)	None identified: no documented boundary or escalation map for accreted functions	Code — role clarity	MEDIUM-HIGH — sustained; drives blame absorption	Documented boundaries + escalation (P1, P4); decision log/handover (P8)
5	Poor organisational change management	Process and access changes made without consulting affected operators; effects discovered live	None identified	Code — change management; ss 47–49	MEDIUM-HIGH	Consultation before change, systematised (P2)
6	Inadequate reward and recognition	No development pathway within the room; competence and responsibility unrecognised	None identified: no progression or recognition structure exists for the room	Code — reward and recognition	MEDIUM — sustained; retention risk	Rotating supervisor function with in-charge recognition and trained pathway (P6)
7	Poor organisational justice	Complaints and workplace matters raised without acknowledgment, outcome or feedback; inconsistent process standards	None identified: no response standard; cl 7.1.11 feedback not practised	Code — organisational justice; cl 7.1.11 (agreement)	HIGH — sustained; injury of this kind has occurred	Complaint-response standard (P4); one rulebook (P5); written feedback per matter
8	Remote/isolated work (after hours)	Nights/weekends worked without present supervision while all functions continue	None identified	Code — remote/isolated work	HIGH after-hours	24/7 on-shift supervisor (P6); handover continuity (P8)
9	Distressed and aggressive callers	First point of contact; a substantial proportion of contacts in distress, fear, conflict or crisis; accusations directed at the operator	None identified: no debrief/defusing step, no post-call recovery arrangement	s 19; Code — violence/aggression (verbal)	HIGH — daily, cumulative	Triage end-point + escalation (P1, P4); debrief and recovery arrangements (scope (a), (b))

#	HAZARD (Code)	HOW IT PRESENTS	EXISTING CONTROLS IDENTIFIED	REQUIRED BY	RISK (duration × frequency × severity)	PROPOSED CONTROL
10	Fatigue	Rostering capable of sub-minimum breaks; team-wide pattern of fatigue absorbed through personal leave on/after high-volume periods — visible in rosters read against leave and demand data	Informal only: workers' own personal leave operating as the de facto fatigue control	Award/FRMS minimum breaks; s 19; Code	HIGH — a sub-minimum break has occurred and been found by the Regulator's reviewing officer to be unreasonable management action	Fatigue rules built into roster design — recovery provided by the roster, not purchased by the worker (P7)

PART 3 — WHAT THE MATRIX DISCLOSES

Read as a whole, the matrix records a single consistent position: each identified hazard has been operating without documented identification, without assessment, without controls selected through the hierarchy, and without the consultation sections 47–49 require — over a sustained period, in a room performing a patient-safety function, in which a psychological injury of the kind these hazards produce has already occurred. The only control identified anywhere in the matrix is informal and worker-funded (personal leave operating as the de facto fatigue control). This is recorded factually, from the operator's vantage point, and is exactly the question the clause 7.2 assessment exists to examine: the assessment can confirm, correct, or complete this matrix — and I ask that it do so with the operators consulted.

PART 4 — THE PROPOSED CONTROLS, INTEGRATED

P1 Patient-safety design (ownership and backup for on-call/paging accuracy; workable directory pathway; defined escalation) · **P2** Consultation before operational change · **P3** The rotational integrated manager (in hours, on the phones, in the room) · **P4** Complaint-response standard (1 business day acknowledgment / 3 days response / automatic escalation) · **P5** One rulebook for the room · **P6** The 24/7 rotating on-shift supervisor — authority follows the clock; real-time decisions consulted with the room; recognition and a trained pathway · **P7** Evidence-based rostering — coverage from the demand data, fatigue rules built into design, equity visible · **P8** Structured handover and decision log. Implementation is from existing resources except the P6 allowance; each control maps to the matrix rows above; controls to be selected and sequenced in consultation, with owners, dates and review.

PART 5 — THE EIGHT PHASES PROPOSED FOR THE ASSESSMENT

- 1. GOVERNANCE.** An assessor independent of the functions being assessed; published terms of reference; a small steering group including an operator representative, any Health and Safety Representative, and the union. The Code of Practice 2022 and ISO 45003 stated as the methods frame.
- 2. THE RECORDED DATA (work as recorded).** Call volumes by hour and day; emergency-code activity; the rosters audited against the fatigue rules; the team's leave usage mapped against the rosters and the demand data; turnover, vacancy and backfill data; the complaints correspondence; EAP usage; incident history; overtime.
- 3. THE SURVEY (work as experienced).** The "People at Work" instrument — the validated, benchmarked psychosocial risk survey developed with the Queensland regulator, available without cost — offered to all operators with anonymity protected.
- 4. OBSERVATION (work as done).** The assessor attends the room across shift types, including night and weekend shifts, and observes the work as it is actually performed — the concurrency, the interruptions, the emergency activations.
- 5. INTERVIEWS.** An interview offered to every operator; structured prompts; and exit interviews sought from operators who have recently left the area.
- 6. CONVERGENCE AND RISK RATING.** The data, survey, observation and interview findings triangulated; each hazard rated by duration, frequency and severity as the Code provides; the resulting risk register shared with the operators.
- 7. CONTROL SELECTION IN CONSULTATION.** A controls workshop with the operators; controls considered in the order the Regulation requires; each adopted control given an owner and an implementation date. The proposed controls at Part 4 are offered as a starting set.
- 8. REVIEW.** Implemented controls carry review dates; a small set of indicators is tracked (fatigue-rule compliance, complaint response times, leave-against-demand correlation); the survey repeated at twelve months.

PART 6 — MATTERS PROPOSED FOR INCLUSION IN SCOPE

- (a)** A critical-incident debrief and peer-support step following defined trigger events (emergency codes, deaths, abusive or distressing calls), as is standard in comparable emergency communications settings.
- (b)** Post-call recovery arrangements — a short off-queue recovery after distressing calls — and a defined surge protocol for periods when demand exceeds capacity.
- (c)** The physical work environment of the room: headset acoustic protection, noise, console ergonomics, screen-based work breaks, and lighting.

- (d) A quality-assurance framework including call-recording review, introduced in consultation and framed as protective — an objective record that resolves complaints and accusations without them resting on the operator.
- (e) An evidence-based occupancy standard, so that sustained utilisation remains within recognised bounds for call-handling work.
- (f) Service standards on departments for the supply of roster and contact-detail updates to the Switchboard, addressing the recurring cause of misdirected calls at its source.
- (g) A documented training and competency framework for the room's functions, including any shift-supervision function.
- (h) A standing, anonymous route for operators to raise hazards, with feedback guaranteed, measured at the twelve-month review.

PART 7 — THE CONDUCT OF THE ASSESSMENT (worker recommendations)

Independence of the assessor. It is recommended that the assessor be external to the health service, led by registered organisational psychologists with work health and safety capability, without prior engagement with the health service on this subject matter, and with a reporting line to a level of the organisation above the functions being assessed. Findings reported to the functions under assessment cannot serve the review step the Regulation requires.

Observation protocol. Observation under Phase 4 is recommended to be conducted across the full operating cycle — day, evening, night and weekend shifts — with attendance unannounced within an agreed assessment window, under a written protocol settled in consultation before observation begins, recording at minimum: call volumes and concurrency, single-operator periods, interruptions, escalation attempts and their response times, emergency activations, and handover practice. An assessment of a 24-hour room conducted in business hours only assesses a different room.

The survey instrument. "People at Work" is nominated as the Phase 3 instrument: it is validated, benchmarked across industries, developed with WorkSafe Queensland, and available without cost (peopleatwork.gov.au). Anonymity protected; all operators offered participation.

Interview themes. It is recommended the Phase 5 interviews include, at minimum: what a difficult shift looks like and what happens afterwards; whom the operator would tell about a hazard and what happened on the last occasion they did; and what, if anything, operators have stopped reporting and why. The last question is the standard test for normalisation in high-exposure work environments and is best asked by an independent assessor.

Indicators and review. A small leading-indicator set tracked from completion: fatigue-rule compliance in rosters as published; complaint acknowledgment and response times; the leave-against-demand correlation; and the survey repeated at twelve months, with results shared with the room.

PART 8 — RECOMMENDED ASSESSOR (providers available)

Suitably qualified independent providers are available locally; the following are put forward to show availability and to assist scoping. I have no association with any provider named; selection of the assessor is itself a matter for consultation under section 48, against the independence criteria at Part 7.

RECOMMENDED	Altius Group — integrated teams of organisational psychologists, work health and safety consultants and allied health professionals; national provider with established health-sector practice; psychosocial risk assessment is an established service line.	Level 11, 231 North Quay, Brisbane QLD 4000 · 1800 258 487 · altius.au
ALTERNATIVE	Psych Group — psychosocial risk assessment practice led by registered psychologists with specialist expertise in psychosocial risk and psychological injury; Brisbane-based.	9/204 Alice Street, Brisbane QLD 4000 · 1300 611 993 · admin@psychgroup.com.au · psychgroup.com.au
INSTRUMENT	People at Work — the validated psychosocial risk survey developed with WorkSafe Queensland; no cost; nominated as the Phase 3 instrument whichever assessor is engaged.	peopleatwork.gov.au
ALSO AVAILABLE	Support Safety Solutions (Brisbane) — psychosocial risk assessments and WHS system development aligned to ISO 45001 · Psychosocial Risk Strategies — assessments and deliverables aligned to the WHS Act, Regulation, Code and ISO 45003 · SHEQ Management — certified ISO 45003 lead auditors, structured six-step assessment framework. Named to demonstrate the availability of qualified providers; any provider meeting the Part 7 criteria is acceptable.	supportsafetysolutions.com.au · psychriskstrategies.com.au · sheqmanagement.com.au

PART 9 — CONSULTATION

This package is offered as an input to the scoping of the assessment under ss 47–49. I ask that the scoping discussion address each matrix row, each phase, each scope item, the conduct recommendations at Part 7 and the selection of the assessor, with the outcome on each recorded in the consultation record. I will attend any discussion at any time, with a support person and my union representative present.

Cory Lea Shepherd · AO3, Switchboard Services, Logan Hospital · 0417 400 227 · coryshepherd1@hotmail.com